

1. The midwife is assessing a pregnant client with a diagnosis of abruptio placentae. The midwife is expected to note the following manifestations EXCEPT?
 - A. Abdominal and low back pain
 - B. Painless vaginal bleeding**
 - C. Uterine irritability
 - D. Uterine tenderness
2. A pregnant client tells the midwife that she felt wetness on her peripad and found some clear fluid. The midwife inspects the perineum and notes the presence of the umbilical cord. What is the immediate midwifery action?
 - A. Monitor the fetal heart rate.
 - B. Notify the primary health care provider.
 - C. Transfer the client to the delivery room.
 - D. Place the client in the Trendelenburg position.**
3. A prenatal client has been diagnosed with a vaginal infection from the organism *Candida albicans*. What should the midwife expect to note on assessment of the client?
 - A. Costovertebral angle pain.
 - B. Absence of any observable signs.
 - C. Pain, itching, and vaginal discharge.**
 - D. Proteinuria, hematuria, and hypertension.
4. A prenatal client has a suspected diagnosis of iron deficiency anemia. On assessment, which finding should the midwife expect to note as a result of this condition?
 - A. Dehydration
 - B. Overhydration
 - C. A high hematocrit levels
 - D. Difficulty in breathing**
5. A client diagnosed with gestational hypertension has just been admitted and is in early active labor. Which assessment finding should the midwife most likely expect to note?
 - A. Increased urine output.
 - B. Increased blood pressure.**
 - C. Decreased fetal heart rate.
 - D. Decreased brachial reflexes.
6. The midwife in the prenatal clinic is monitoring a client who is pregnant with twins. The midwife monitors the client closely for which priority complication that is associated with a twin pregnancy?
 - A. Costovertebral angle tenderness
 - B. Hemorrhoids
 - C. Maternal anemia**
 - D. Postterm labor

7. A clinic midwife is assessing a prenatal client who has been diagnosed with heart disease. The midwife carefully assesses the client's vital signs, weight, and fluid and nutritional status to detect for complications caused by which pregnancy-related concern?
- A. Rh incompatibility
 - B. Fetal cardiomegaly
 - C. The increase in circulating blood volume
 - D. Hypertrophy and increased contractility of the heart
8. A client in labor has a diagnosis of sickle cell anemia. Which action will the midwife take to assist in preventing the client from experiencing a sickling crisis during labor?
- A. Being reassuring
 - B. Administering oxygen
 - C. Preventing bearing down
 - D. Maintaining strict asepsis
9. The midwife is administering magnesium sulfate to a client experiencing severe preeclampsia. What intervention should the midwife implement during the administration of magnesium sulfate for this client?
- A. Schedule a daily ultrasound to assess fetal movement.
 - B. Schedule a nonstress test every 4 hours to assess fetal well-being.
 - C. Assess the client's temperature every 2 hours because the client is at high risk for infection.
 - D. Assess for signs and symptoms of labor since the client's level of consciousness may be altered.
10. During a routine prenatal visit, a client in her third trimester of pregnancy reports having frequent calf pain when she walks. The midwife suspects superficial thrombophlebitis and checks for which sign associated with this condition?
- A. Severe chills
 - B. Kernig's sign
 - C. Brudzinski's sign
 - D. Palpable hard thrombus
11. A clinic midwife is caring for a client with a suspected diagnosis of gestational hypertension. The midwife assesses the client, expecting to note which set of findings if gestational hypertension is present?
- A. Edema, ketonuria, and obesity.
 - B. Edema, tachycardia, and ketonuria.
 - C. Glycosuria, hypertension, and obesity.
 - D. Elevated blood pressure and proteinuria.
12. The midwife has admitted a client diagnosed with gestational hypertension who is in labor. The midwife monitors the client closely for which complication of gestational hypertension?
- A. Seizures
 - B. Hallucinations
 - C. Placenta previa

- D. Altered respiratory status
13. The hemoglobin levels of a client in her first trimester of pregnancy are indicative of iron deficiency anemia. Which assessment findings support the diagnosis of this type of anemia?
- A. Yellowish sclera.
 - B. Reports of severe fatigue.
 - C. Pink mucous membranes.
 - D. Increased vaginal secretions.
14. A prenatal client is being evaluated for possible gestational diabetes. Which data identified and documented after the client's initial nursing assessment would support that diagnosis?
- A. 22 years old.
 - B. A gravida 4, para 0, aborta 3.
 - C. 5 inches to 6 inches ' tall, weighs 59kg.
 - D. Stated, "I get really tired after working all day".
15. The midwife is caring for a client diagnosed with preeclampsia. When the client's condition progresses from preeclampsia to eclampsia, what should the midwife's first action be?
- A. Maintain an open airway.
 - B. Administer oxygen by face mask.
 - C. Assess the maternal blood pressure and fetal heart tones.
 - D. Administer an intravenous infusion of magnesium sulfate.
16. A pregnant client is diagnosed with partial placenta previa. In explaining the diagnosis, the midwife tells the client that the usual treatment for partial placenta previa is which of the following?
- A. Activity limited to bed rest.
 - B. Platelet infusion.
 - C. Immediate cesarean delivery.
 - D. Labor induction with oxytocin.
17. Which of the following would the midwife most likely expect to find when assessing a pregnant client with abruption placenta?
- A. Excessive vaginal bleeding
 - B. Rigid, board-like abdomen
 - C. Tetanic uterine contractions
 - D. Premature rupture of membranes
18. A client tells the midwife, "I think my baby likes to hear me talk to him." When discussing neonates and stimulation with sound, which of the following would the midwife include as a means to elicit the best response?
- A. High-pitched speech with tonal variations.
 - B. Low-pitched speech with a sameness of tone.
 - C. Cooing sounds rather than words.
 - D. Repeated stimulation with loud sounds.

19. A pregnant patient asks the midwife if she can take castor oil for her constipation. How should the midwife respond?
- A. "Yes, it produces no adverse effect."
 - B. "No, it can initiate premature uterine contractions."
 - C. "No, it can promote sodium retention."
 - D. "No, it can lead to increased absorption of fat-soluble vitamins."
20. A patient in her 14th week of pregnancy has presented with abdominal cramping and vaginal bleeding for the past 8 hours. She has passed several clots. What is the primary nursing diagnosis for this patient?
- A. Knowledge deficit
 - B. Fluid volume deficit.
 - C. Anticipatory grieving.
 - D. Pain.
21. A patient with pregnancy-induced hypertension probably exhibits which of the following symptoms?
- A. Proteinuria, headaches, vaginal bleeding
 - B. Headaches, double vision, vaginal bleeding
 - C. Proteinuria, headaches, double vision
 - D. Proteinuria, double vision, uterine contractions
22. A client makes a routine visit to the prenatal clinic. Although she is 14 weeks pregnant, the size of her uterus approximates that in an 18- to 20-week pregnancy. Dr. Charles diagnoses gestational trophoblastic disease and orders ultrasonography. The midwife expects ultrasonography to reveal:
- A. An empty gestational sac.
 - B. Grapelike clusters.
 - C. A severely malformed fetus.
 - D. An extrauterine pregnancy.
23. A 39-year-old at 37 weeks gestation is admitted to the hospital with complaints of vaginal bleeding following the use of cocaine 1 hour earlier. Which complication is most likely causing the client's complaint of vaginal bleeding?
- A. Placenta previa
 - B. Abruptio placentae
 - C. Ectopic pregnancy
24. A client with type 1 diabetes mellitus who is a multigravida visits the clinic at 27 weeks gestation. The midwife should instruct the client that for most pregnant women with type 1 diabetes mellitus:
- A. Weekly fetal movement counts are made by the mother.
 - B. Contraction stress testing is performed weekly.
 - C. Induction of labor begins at 34 weeks' gestation.
 - D. Nonstress testing is performed weekly until 32 weeks' gestation.

25. When administering magnesium sulfate to a client with preeclampsia, the midwife understands that this drug is given to:
- A. Prevent seizures.
 - B. Reduce blood pressure.
 - C. Slow the process of labor.
 - D. Increase diuresis.
26. A primigravida client at about 35 weeks gestation in active labor has had no prenatal care and admitted to cocaine use during the pregnancy. Which of the following persons must the midwife notify?
- A. Nursing unit manager so appropriate agencies can be notified.
 - B. Head of the hospital's security department.
 - C. Chaplain in case the fetus dies in utero.
 - D. Physician who will attend the delivery of the infant.
27. A client with eclampsia begins to experience a seizure. Which of the following would the midwife in charge do first?
- A. Pad the side rails.
 - B. Place a pillow under the left buttock.
 - C. Insert a padded tongue blade into the mouth.
 - D. Maintain a patent airway.
28. Which of the following is the most common kind of placental adherence seen in pregnant women?
- A. Accreta
 - B. Placenta previa
 - C. Percreta
 - D. Increta
29. With regard to small-for-gestational-age (SGA) infants and intrauterine growth restriction (IUGR), midwives should be aware that:
- A. In the first trimester, diseases or abnormalities result in asymmetric IUGR.
 - B. Infants with asymmetric IUGR have the potential for normal growth and development.
 - C. In asymmetric IUGR, weight is slightly more than SGA, whereas length and head circumference are somewhat less than SGA.
 - D. Symmetric IUGR occurs in the later stages of pregnancy.
30. Heartburn and flatulence, common in the second trimester, are most likely the result of which of the following?
- A. Increased plasma HCG levels
 - B. Decreased intestinal motility
 - C. Decreased gastric acidity
 - D. Elevated estrogen levels
31. A woman at 10 weeks' gestation is diagnosed with gestational trophoblastic disease (hydatiform mole). Which of the following findings would the nurse expect to see?
- A. Platelet count of 550,000/ mm³
 - B. Dark brown vaginal bleeding

- C. White blood cell count 17,000/ mm³
 - D. Macular papular rash
32. You are caring for a patient who was admitted to labor and delivery at 32 weeks' gestation and diagnosed with preterm labor. She is currently on magnesium sulfate, 2 gm per hour. Upon your initial assessment you note that she has a respiratory rate of 8 with absent deep tendon reflexes. What will be your first nursing intervention?
- A. Elevate head of the bed
 - B. Notify the obstetrician
 - C. Discontinue magnesium sulfate
 - D. Draw a serum magnesium level
33. A primiparous woman has been admitted at 35 weeks' gestation and diagnosed with HELLP syndrome. Which of the following laboratory changes is consistent with this diagnosis?
- A. Hematocrit dropped to 28%.
 - B. Platelets increased to 300,000 cells/mm³.
 - C. Red blood cells increased to 5.1 million cells/mm³.
 - D. Sodium dropped to 132 mEq/dL.
34. According to agency policy, the perinatal nurse provides the following intrapartal nursing care for the patient with preeclampsia:
- A. Take the patient's blood pressure every 6 hours.
 - B. Encourage the patient to rest on her back.
 - C. Notify the physician of a urine output greater than 30 mL/hr.
 - D. Administer magnesium sulfate according to agency policy
35. The single most important risk factor for preterm birth includes:
- A. Uterine and cervical anomalies
 - B. Infection
 - C. Increased BMI
 - D. Prior preterm birth
33. The nurse is caring for a woman at 28 weeks' gestation with a history of preterm delivery. Which of the following laboratory data should the nurse carefully assess in relation to this diagnosis?
- A. Human relaxin levels
 - B. Amniotic fluid levels
 - C. Alpha-fetoprotein levels
 - D. Fetal fibronectin levels
34. For the patient with which of the following medical problems should the nurse question a physician's order for beta agonist tocolytics?
- A. Type 1 diabetes mellitus
 - B. Cerebral palsy
 - C. Myelomeningocele
 - D. Positive group B streptococci culture

35. A woman in labor and delivery is being given subcutaneous terbutaline for preterm labor. Which of the following common medication effects would the nurse expect to see in the mother?
- A. Serum potassium level increases
 - B. Diarrhea
 - C. Urticaria
 - D. Complaints of nervousness
36. Which of the following signs or symptoms would the nurse expect to see in a woman with concealed abruptio placentae?
- A. Increasing abdominal girth measurements
 - B. Profuse vaginal bleeding
 - C. Bradycardia with an aortic thrill
 - D. Hypothermia with chills
37. The nurse working in a prenatal clinic is providing care to three primigravida patients. Which of the patient findings would the nurse highlight for the physician?
- A. 15 weeks, denies feeling fetal movement
 - B. 20 weeks, fundal height at the umbilicus
 - C. 25 weeks, complains of excess salivation
 - D. 30 weeks, states that her vision is blurry
38. The perinatal nurse is assessing a woman in triage who is 34 + 3 weeks' gestation in her first pregnancy. She is worried about having her baby "too soon," and she is experiencing uterine contractions every 10 to 15 minutes. The fetal heart rate is 136 beats per minute. A vaginal examination performed by the health-care provider reveals that the cervix is closed, long, and posterior. The most likely diagnosis would be:
- A. Preterm labor
 - B. Term labor
 - C. Back labor
 - D. Braxton-Hicks contractions
39. Which of the following nursing diagnoses is of highest priority for a client with an ectopic pregnancy who has developed disseminated intravascular coagulation (DIC)?
- A. Risk for deficient fluid volume
 - B. Risk for family process interrupted
 - C. Risk for disturbed identity
 - D. High risk for injury
40. Which of the following laboratory values is most concerning in a client with pregnancy-induced hypertension?
- A. Total urine protein of 200 mg/dL
 - B. Total platelet count of 40,000 mm
 - C. Uric acid level of 8 mg/dL
 - D. Blood urea nitrogen 24 mg/dL
41. A pregnant client with a history of multiple sexual partners is at highest risk for which of the following complications
- A. Ectopic pregnancy

- B. Gestational diabetes
 - C. Pregnancy-induced hypertension
 - D. Premature rupture of membranes
42. A multigravida at 38 weeks' gestation is admitted with painless, bright red bleeding and mild contractions every 7 to 10 minutes. Which of the following assessments should be avoided?
- A. Maternal vital sign
 - B. Fetal heart rate
 - C. Contraction monitoring
 - D. Cervical dilation
43. Which of the following would be the nurse's most appropriate response to a client who asks why she must have a cesarean delivery if she has a complete placenta previa?
- A. "You will have to ask your physician when he returns."
 - B. "You need a cesarean to prevent hemorrhage."
 - C. "The placenta is covering most of your cervix."
44. Which of the following would the nurse assess in a client experiencing abruptio placenta?
- A. Bright red, painless vaginal bleeding.
 - B. Concealed or external dark red bleeding.
 - C. Palpable fetal outline
 - D. Soft and nontender abdomen
45. Which of the following is described as premature separation of a normally implanted placenta during the second half of pregnancy, usually with severe hemorrhage?
- A. Placenta previa
 - B. Ectopic pregnancy
 - C. Incompetent cervix
 - D. Abruptio placenta
46. A client at 24 weeks gestation has gained 6 pounds in 4 weeks. Which of the following would be the priority when assessing the client?
- A. Glucosuria
 - B. Depression
 - C. Hand/face edema
 - D. "The placenta is covering the opening of the uterus and blocking your baby"
47. Which of the following would the nurse identify as a classic sign of Pregnancy Induced Hypertension?
- A. Edema of the feet and ankles
 - B. Edema of the hands and face
 - C. Weight gain of 1 lb/week
 - D. Early morning headache
48. In which of the following types of spontaneous abortions would the nurse assess dark brown vaginal discharge and a negative pregnancy test?
- A. Threatened
 - B. Imminent
 - C. Missed

- D. Incomplete
49. Which of the following factors would the nurse suspect as predisposing a client to placenta previa?
- A. Multiple gestation
 - B. Uterine anomalies
 - C. Abdominal trauma
 - D. Renal or vascular disease
50. Which of the following signs and symptoms would the nurse expect to see in a woman with concealed abruption placentae/
- A. Increasing abdominal girth measurements
 - B. Profuse vaginal bleeding
 - C. Bradycardia with an aortic thrill
 - D. Hypothermia with chills